

Calling It What It Is:

Acknowledging the Trauma of Multi-Systemic Youth Caregiving

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Abstract

Caregivers of multi-system-involved youth often experience chronic strain, emotional distress, and repeated exposure to child-to-parent violence (CPV); yet their trauma remains largely unrecognized within service systems. This commentary draws from lived experience, practitioner insights, and a review of over forty scholarly articles to argue that caregiver strain in high-conflict, multi-system contexts must be understood as trauma, not merely stress or burnout. The failure to name caregiver trauma has led to misdiagnoses, inadequate interventions, and widespread systemic invalidation. By reframing caregiver experiences through a trauma-informed lens, this paper calls for structural changes in assessment, service delivery, and professional training. The commentary further proposes the concept of *Trauma and Relational Adversity in Caregiver Environments (TRACEs)* as a parallel to Adverse Childhood Experiences (ACEs), offering a framework for addressing the unacknowledged emotional and systemic burdens caregivers carry. Naming this trauma is not a semantic shift. It is an essential intervention. Without this recognition, we risk the collapse not just of families, but of the systems designed to support them.

Keywords: caregiver trauma, child-to-parent violence, multi-system youth, family systems, trauma-informed care, Adverse Caregiver Experiences

Notes

Author's Note

This paper was independently researched and written by the author. It does not represent a formal study involving human subjects and does not require IRB approval. No conflicts of interest are declared.

Considering Intersectionality and Youth Voice

The experience of caregiving is not monolithic. Caregivers from Black, Indigenous, immigrant, and LGBTQIA+ communities often face compounded harms due to systemic racism, cultural stigma, and institutional distrust. These intersecting oppressions heighten the emotional and structural strain of caregiving.

Likewise, while this paper centers caregivers, it is vital to recognize that youth behavior often emerges from their own trauma, misdiagnosis, or systemic neglect. Including youth perspectives in system reform efforts fosters mutual healing and relational restoration.

Introduction: When the System Fails the Caregiver

In every high-stakes crisis involving a child, there is someone just out of frame; absorbing the chaos, holding the pieces, and often breaking in silence. For caregivers of multi-system-involved youth, the burden is not merely emotional or logistical. It is traumatic (Solomon & Heide, 2005; Liu et al., 2020).

These are not isolated parenting challenges. They are compounded, persistent stressors intensified by systemic fragmentation, emotional isolation, and, too often, child-to-parent violence (CPV) (Condry & Miles, 2013; O'Toole et al., 2020; Toole-Anstey et al., 2023). Yet the dominant narrative still frames this experience as burnout, poor parenting, or secondary stress. The result? A caregiver left misdiagnosed, misunderstood, and unsupported by the very systems meant to offer help (Holt & Shon, 2016; Baker-Ericzén et al., 2010).

This paper challenges that framing. Drawing on lived experience, peer-practice insights, and a review of over 40 scholarly articles, it makes the case that caregiving for a multi-system-involved youth must be understood through the lens of trauma; not the child's alone, but the caregiver's too (Accurso et al., 2014; Duppong Hurley et al., 2016; McCarthy et al., 2016).

It argues that when CPV is present, when system barriers stack, and when support is absent, what we're witnessing is not stress, it's symptomatic collapse. And if we fail to recognize and respond to that trauma, we lose not just the caregiver, but the entire care infrastructure around the child (Toole-Anstey et al., 2023; Curtis et al., 2025).

The aim of this paper is to:

- Name the invisible trauma caregivers endure
- Connect CPV and caregiver strain to broader systemic failures
- Propose family-centered, trauma-informed solutions that address the full scope of harm

Until we center the caregiver as a potential trauma survivor, not just a collateral participant, we will continue to misdiagnose family crises, mistreat their causes, and misplace our interventions.

It's time to call it what it is.

From Stress to Trauma: The Neurological and Emotional Toll

For caregivers of multi-system-involved youth, the emotional burden often exceeds the threshold of manageable stress and enters the realm of trauma. While terms like "caregiver fatigue," "burnout," and "strain" are commonly used, they are insufficient to describe the physiological, psychological, and neurobiological consequences caregivers endure under prolonged, unresolved stress (Solomon & Heide, 2005; Liu et al., 2020).

What Trauma Does to the Brain

Chronic exposure to trauma, whether direct or secondary, leads to measurable changes in the brain. Trauma impacts areas responsible for emotional regulation and executive functioning, including the amygdala, hippocampus, and prefrontal cortex (Solomon & Heide, 2005; Figley, 2012). These areas govern how individuals respond to threats, process fear, and maintain a sense of safety. When overstimulated over time, the brain begins to operate in a constant state of hypervigilance, impairing the ability to respond rationally to stressors.

Caregivers living in high-stress environments, particularly those facing child-to-parent violence (CPV), experience sustained activation of the sympathetic nervous system. This chronic “fight-or-flight” state often presents as anxiety, emotional dysregulation, dissociation, or physical illness (McEwen, 2017). Yet caregivers in these states are rarely assessed for trauma exposure, especially when the threat comes from within their own home.

The Invisible Toll of Persistent Dysregulation

As neurobiological stress responses go unchecked, caregivers may exhibit symptoms of complex trauma, including difficulty trusting professionals, emotional numbness, memory lapses, and social withdrawal (Solomon & Heide, 2005). Their trauma is compounded by institutional invalidation, being dismissed, blamed, or told they are overreacting to their child's behavior, leading to further distress and disengagement from care systems (Baker-Ericzén et al., 2010).

When systems treat caregiver strain as merely a side effect of youth behavior, they ignore the caregiver as a trauma-affected individual. This blind spot leads to inappropriate referrals, missed diagnoses, and a lack of meaningful interventions for those expected to hold the family system together.

Why This Distinction Matters

Labeling caregiver distress as “burnout” implies the solution is rest. But labeling it as trauma suggests the solution must be healing, support, and safety. This distinction is not semantic; it is strategic. It determines whether caregivers receive validation, trauma-informed services, and relational repair, or are simply advised to “try harder.”

Until the trauma of caregiving in complex family systems is named and treated as such, the risk of caregiver collapse remains dangerously high, not only for the individual but for the stability of the youth and systems surrounding them.

The Invisible Reversal: Understanding Child-to-Parent Violence (CPV)

Child-to-parent violence (CPV) is one of the most misunderstood and under-addressed forms of family violence. Unlike other interpersonal violence dynamics, CPV inverts traditional power hierarchies. The caregiver, typically seen as the authority figure, becomes the target of

aggression by the child. This role reversal not only defies cultural assumptions but disrupts how professionals assess risk, assign blame, and intervene (Condry & Miles, 2013; Holt & Shon, 2016).

Prevalence and Complexity

Despite growing concern, CPV remains difficult to define and measure. Prevalence estimates vary widely: physical violence rates range from 5% to 21%, while verbal or psychological abuse affects up to 93% of families studied (Rutter, 2023; Beckmann et al., 2017). Emotional abuse is both the most common and the least consistently defined, often overlapping with verbal aggression, intimidation, and manipulative control (Cristian Suárez-Relinque et al., 2022).

Many caregivers report multiple forms of CPV simultaneously, including insults, threats, destruction of property, coercive control, and, in rare but serious cases, physical injury. These behaviors are often chronic, beginning in early adolescence and escalating over time; especially in youth with trauma histories, neurodevelopmental disorders, or unaddressed emotional regulation challenges (Arias-Rivera et al., 2021; Harries et al., 2025).

Risk Factors and Power Inversion

CPV is not a single-issue problem. Risk factors span individual (e.g., trauma exposure, mental illness), relational (e.g., insecure attachment, harsh discipline), and systemic levels (e.g., housing instability, fragmented services) (Junco-Guerrero et al., 2023; Curtis et al., 2025). Substance use by either the child or parent further increases risk, often triggering or exacerbating violent incidents.

Yet unlike other forms of family violence, CPV is rarely taken seriously by service providers or legal systems. Caregivers often hesitate to report it due to shame, fear of retaliation, or fear of losing custody (Toole-Anstey et al., 2023). This silence not only protects the child, but isolates the caregiver in escalating, traumatic conditions.

A Problem Hidden in Plain Sight

Because CPV doesn't align with dominant cultural narratives about victimhood, it remains hidden in plain sight. Parents may describe living with daily fear, walking on eggshells in their own home, and enduring physical or emotional abuse without language to name it, or support to address it (O'Toole et al., 2020).

When they do speak out, caregivers are often dismissed as incompetent, or worse, blamed for provoking the violence. Without a framework that recognizes CPV as legitimate family trauma, caregivers are left to navigate these crises alone.

Legal Risk and Structural Silencing

Reporting child-to-parent violence is rarely straightforward. Caregivers who disclose aggression by their child may face unintended legal consequences, including investigations from child

protective services, accusations of parental neglect, or loss of custody. In some jurisdictions, mandatory reporting laws require professionals to escalate these disclosures without fully understanding the context; effectively penalizing the caregiver for seeking help. This is particularly true when situations necessitate the involvement of law enforcement. This creates a chilling effect, where silence becomes a survival strategy, and the cycle of trauma continues unaddressed.

The Myth of the Bad Parent: Stigma, Silence, and Systemic Harm

When child-to-parent violence occurs, society often leaps to one conclusion: the parent must have done something wrong. This myth, rooted in outdated ideas of discipline, control, and family hierarchy, perpetuates a stigma that silences caregivers and distorts the system's response.

Caregivers who disclose CPV are frequently met with skepticism or blame. Service providers may assume the parent is enabling the behavior, failing to set boundaries, or lacking proper discipline skills (Baker-Ericzén et al., 2010; O'Toole et al., 2020). As a result, they are often excluded from treatment plans, judged harshly in courtrooms, or subjected to unrealistic parenting expectations by professionals who lack training in trauma-informed, family-centered care.

Stigma Becomes a Barrier to Help-Seeking

In a narrative inquiry study, caregivers described the experience of seeking help as demoralizing and shaming. Many were told that reporting violence could lead to involvement from child welfare authorities or accusations of parental failure (Toole-Anstey et al., 2023). This produces a paralyzing dilemma: speak up and risk judgment, or stay silent and risk escalation.

Stigma is not just emotional. It is structural. It is embedded in eligibility criteria, risk assessment tools, intake forms, and even diagnostic frameworks that fail to name caregiver trauma or recognize the complexity of CPV (Arias-Rivera et al., 2021). Without appropriate language, caregivers are rendered invisible; unable to articulate their experience in ways that systems will validate.

Trauma and Relational Adversity in Caregiver Environments (TRACES): A Missing Framework

To shift this paradigm, we need more than trauma-informed rhetoric; we need trauma-informed infrastructure. Just as Adverse Childhood Experiences (ACEs) reshaped the landscape of pediatric care, it is time to develop a parallel framework for caregivers. *Adverse Caregiver Experiences* (also ACEs) would account for the chronic, system-driven, and relational trauma endured by those supporting multi-system youth.

This includes:

- Exposure to verbal or physical CPV
- Systemic invalidation or punitive responses
- Persistent emotional labor under threat
- Lack of respite, support, or peer connection
- Institutional gaslighting by providers or authorities

By failing to measure these experiences, we reinforce the idea that caregiver distress is a reflection of weakness; not a symptom of accumulated, unacknowledged harm.

Multi-System Youth, Multi-Layered Strain

Caring for a young person involved in multiple systems(juvenile justice, child welfare, mental health, special education) means navigating a labyrinth of acronyms, appointments, and expectations. For caregivers, the toll of this role isn't just emotional or logistical. It's structural, persistent, and compounded.

The literature is clear: multi-system involvement dramatically increases the strain on caregivers (Accurso et al., 2014; Fawley-King et al., 2020). It introduces fragmented communication, inconsistent eligibility criteria, and conflicting service plans that the caregiver is expected to coordinate and comply with. When combined with child behavioral challenges or CPV, this strain often becomes unmanageable and chronic (McCarthy et al., 2016; Brannan & Heflinger, 2001).

Understanding Caregiver Strain

Caregiver strain is defined as the perceived negative impact of caregiving on emotional, physical, financial, and social functioning (Liu et al., 2020). It includes:

- Objective strain: time lost, financial burden, disrupted routines
- Subjective internalized strain: guilt, fear, helplessness
- Subjective externalized strain: frustration, resentment, irritability (Frank et al., 2016)

Importantly, this strain is not equal across caregivers. Biological parents, foster parents, kinship caregivers, and adoptive parents report different levels of strain depending on the support provided and the behavior of the youth in their care (Fawley-King et al., 2020). However, a consistent theme across all family types is that the greater the system involvement, the higher the caregiver burden.

Strain and Mental Health Outcomes

High levels of caregiver strain are associated with worsened mental health outcomes for both the youth and the caregiver (Williams et al., 2018). Strain predicts lower engagement in services, less effective parenting, and greater likelihood of out-of-home placement, particularly when CPV is present or mental health needs go unmet (Unrau & Grinnell, 2005).

Additionally, caregivers experiencing high strain are less likely to participate in support programs, especially if they perceive those programs to be judgmental or disconnected from their lived experience (Duppong Hurley et al., 2016). This creates a vicious cycle: strain reduces support-seeking, and lack of support increases strain.

The Unacknowledged Role of System Burden

Professionals often interpret caregiver disengagement as noncompliance or resistance. In reality, many caregivers are overwhelmed, isolated, and distrustful; not because they are unwilling, but because the system itself is exhausting and retraumatizing (Baker-Ericzén et al., 2010; Grey et al., 2021). Without integrated, trauma-informed approaches that recognize and address caregiver strain, the most vulnerable families are left unsupported at the very moment they need the most help.

Naming as Intervention: Why It Matters Now

Trauma-informed care often begins with a simple act: naming what has happened. For caregivers of multi-system youth, this naming has rarely extended to them. While their children may receive diagnoses, services, and trauma screenings, the caregiver remains unseen; treated as a function of the child's recovery, not a person in need of healing themselves.

But naming is not just an act of empathy. It is an intervention.

When caregivers are offered language to describe their experiences (*trauma, adverse caregiver experiences, relational violence*) they are more likely to seek support, engage in services, and trust professionals (Toole-Anstey et al., 2023; Grey et al., 2021). When those same experiences are minimized as “parenting stress,” they often retreat, become reactive, or fall through systemic cracks.

From ACEs to TRACEs: Expanding the Framework

The widespread adoption of Adverse Childhood Experiences (ACEs) reframed how pediatric trauma is understood and addressed. It provided a common language for discussing complex trauma histories and created a foundation for trauma-informed care across sectors. But that same logic has yet to be applied to caregivers.

The concept of Trauma and Relational Adversity in Caregiver Environments (TRACEs) invites a parallel shift, one that recognizes that caregivers are trauma survivors. Caregivers navigating CPV, system-driven retraumatization, emotional exhaustion, and institutional betrayal are not “difficult parents.” They are experiencing prolonged exposure to threat, loss, and helplessness (Solomon & Heide, 2005; Liu et al., 2020).

TRACEs

A framework designed to parallel Adverse Childhood Experiences (ACEs), TRACEs captures the systemic, relational, and emotional trauma caregivers endure while supporting multi-system youth.

Adverse Childhood Experiences(ACEs)	Trauma and Relational Adversity in Caregiver Environments (TRACEs)
Emtional/Psychological abuse	Verbal degradation, slander from their child; shame and humiliation in their community; property damage by their child
Physical abuse	Child-to-Parent Violence (CPV)
Parental Mental Illness	Navigating child serving mental/behavioral health systems
Parental/Household member incarceration	Incarceration of a child in Juvenile Justice; responsibility of caregiver to follow through on sentencing requirements and probation
Loss/Separation from a caregiver	Loss/Separation from a child through child welfare or out of home treatment
Household instability/poverty	Instability due to job loss from caregiving; expenses from uninsured, specialized treatment options
Substance use in the household	Child with substance use disorder
Parental separation/divorce	Partner separation/divorce due to stress on relationship, finances, and/or sibling safety
Istitutional betrayal/harm	Institutional gaslighting, denial of services, delays in receiving care

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Naming Changes Response

What we name shapes what we measure. What we measure shapes what we fund, train for, and build interventions around. If caregiver trauma remains unnamed, then:

- It is not screened for
- It is not supported
- It is not treated

Without a shared language for these experiences, systems will continue to offer fragmented or punitive responses to families in crisis. Naming caregiver trauma shifts the narrative from “noncompliance” to survival; from “bad parenting” to unacknowledged harm.

It’s not just semantics; it’s the first step in system transformation.

Toward Family-Centered, Trauma-Informed Systems

Acknowledging caregiver trauma is not just about reframing stories; it’s about rebuilding systems. If we are serious about preventing harm, reducing out-of-home placements, and improving outcomes for multi-system youth, we must center the caregiver not only as a resource but as someone worthy of care in their own right.

What a Trauma-Informed System Must Include

To move from awareness to action, the following shifts are necessary:

1. Caregiver-Inclusive Assessment and Screening

- System intake processes should screen for caregiver trauma, not just child risk.
- Tools must capture the emotional, physical, and systemic burdens caregivers carry (Brannan & Heflinger, 2001; Accurso et al., 2014).

2. Peer-Led and Narrative-Based Interventions

- Caregivers benefit most from relatable, non-judgmental support, often best delivered through peer support models (Duppont Hurley et al., 2016).
- Writing, storytelling, and narrative interventions have shown promise in reducing trauma symptoms among caregivers.

3. Training and Supervision for Professionals

- Professionals need structured training on CPV, caregiver strain, and trauma-responsive engagement.
- Supervision and reflective practices should be built into agency culture to reduce judgment and bias (Arias-Rivera et al., 2021; Baker-Ericzén et al., 2010).

4. Cross-System Collaboration

- Fragmentation is a root cause of family distress.
- Wraparound services, family-focused case management, and multi-agency coordination should be standard, not exceptional (McCarthy et al., 2016; Williams et al., 2018).

5. A Paradigm Shift in Language and Practice

- Replace terms like “treatment-resistant” or “noncompliant” with language that reflects trauma-informed understanding: “*system-fatigued*,” “*trust-breached*,” “*survival-activated*.”
- This change must be mirrored in policies, procedures, and outcome measures.

What This Means for Practice

When caregivers are seen, supported, and engaged as partners, everyone benefits:

- Youth stabilize faster
- Outcomes improve
- System trust is rebuilt

Trauma-informed systems don’t just treat symptoms. They restore dignity to those the system once failed.

When Caregivers Collapse, So Do Systems

Behind every multi-system-involved youth is a caregiver holding the weight of complexity, conflict, and collapse. Their love is not in question. Their capacity is. Not because they lack strength, but because the system keeps asking them to carry what it refuses to name.

This paper has argued that caregiver strain is not just circumstantial. It is often traumatic. It has shown that CPV is misunderstood, caregiver needs are minimized, and systemic structures are ill-equipped to offer sustained support. In naming this, we do not point fingers at families. We call the system into responsibility.

If we are to build trauma-informed care that actually prevents crisis, we must begin where the breakdown begins: with the unsupported caregiver.

This is not just about awareness.

It’s about accountability.

It’s about language, leadership, and legislation.

It’s about building systems that don’t collapse the moment a caregiver does.

Because when the caregiver falls, the whole system follows.

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